

EPINEPHRINE

1:1,000 · 1 MG/ML

SYMPATHOMIMETIC · CATECHOLAMINE

ACTION

Alpha + Beta agonist. Causes **vasoconstriction** (alpha-1), **bronchodilation**, and increased **HR/contractility** (beta-1/2). Reverses the life-threatening effects of anaphylaxis rapidly.

INDICATIONS

- Anaphylaxis
- Severe Allergic Reaction
- Refractory Bronchospasm
- Asthma (refractory)

CONTRAINDICATIONS

- Hypersensitivity
- Hypovolemic Shock
- Coronary insufficiency (relative)
- Hypertension (relative)

DOSE & ROUTE

Adult: 0.3–0.5 mg IM (lateral thigh) — may repeat q5–15 min

Peds: 0.01 mg/kg IM (max 0.3 mg) — lateral thigh

Asthma (Adult): 0.3–0.5 mg SQ

Onset: **1–2 min** · Duration: **5–15 min**

ADVERSE EFFECTS

- Tachycardia
- Hypertension
- Dysrhythmias
- Anxiety / Tremor
- Pallor / Headache
- Angina

PRECAUTIONS

Use caution in elderly, known **cardiac disease**, **HTN**, and **diabetes**. May precipitate angina or MI in susceptible patients.

⚠ CRITICAL DISTINCTION

1:1,000 is IM/SQ only. Never give this concentration IV — cardiac arrest protocol uses **1:10,000 (0.1 mg/mL)**. Double-check your concentration every time.

🔪 PRO TIP

Inject into the **lateral thigh (vastus lateralis)** — faster absorption than deltoid. Don't hesitate: in true anaphylaxis, epi first, everything else second.

AMIODARONE

CLASS III ANTIARRHYTHMIC · MULTI-CLASS

ACTION

Prolongs **action potential** and refractory period. Slows conduction through the **AV node**. Has Class I, II, III, and IV properties — alpha/beta blocking, sodium/calcium channel effects. Broad antiarrhythmic coverage.

INDICATIONS

- V-Fib / Pulseless V-Tach
- Stable V-Tach
- SVT (adenosine-refractory)
- Rapid AFib / AFlutter

CONTRAINDICATIONS

- Sinus Bradycardia
- 2nd / 3rd Degree AV Block
- Cardiogenic Shock
- Iodine Hypersensitivity
- Sick Sinus Syndrome

DOSE & ROUTE

CARDIAC ARREST (V-FIB / PULSELESS V-TACH)

Adult: **300 mg IV/IO rapid push** — 2nd dose 150 mg

Peds: **5 mg/kg IV/IO rapid push**

STABLE V-TACH / SVT / RAPID AFIB

Adult: **150 mg IV over 10 min**, then 1 mg/min infusion

Onset: **Immediate IV** · Half-life: **Weeks**

ADVERSE EFFECTS

- Hypotension ⚠
- Bradycardia
- Heart Block
- Prolonged QT
- Phlebitis (IV site)
- Nausea

PRECAUTIONS

Contains **Polysorbate 80** — major cause of hypotension, especially with rapid infusion. Check thyroid history (iodine-containing). Monitor BP and ECG continuously.

⚠ HYPOTENSION RISK

Hypotension is common — have a **fluid bolus ready** before you push. For stable rhythms, infuse over 10 min — never rapid push unless cardiac arrest.

🔪 PRO TIP

Two very different protocols — **arrest vs. stable**. In arrest: push it fast. In stable: go slow and watch the BP. Know which situation you're in before you reach for the syringe.